Perseverant Digital
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A FIELD GUIDE FROM PERSEVERANT DIGITAL · FOR ELECTIVE HEALTHCARE

The Cost-per-Client Tracking Map

Trace every lead from first click to paying client, find where data and money leak between your tools, and decide whether each fix is a setting, an integration or a small build.

THE SHORT ANSWER · 65 WORDS

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Judge marketing by cost per client, not cost per lead. Map every stage from first click to signed client, note which tool holds each stage, and test whether the lead's source survives the trip. Look for leaks in the handoffs between forms, phones, CRM and job or case software. Fix them with settings first, integrations second, and custom builds only when the numbers justify it.

Why can’t you judge a channel by cost per lead?

Because a lead is not a client. One channel can produce cheap leads that never answer the phone. Another can produce expensive leads that book, show up and sign. Stop measuring at the form fill and budget drifts toward the cheap channel while revenue stays flat.

The number that matters is cost per client, with cost per booked and held appointment close behind. Those numbers live in different tools: the ad platform, the form, the CRM, your system of record. Nobody connects them by default.

Speed leaks too. Harvard Business Review’s 2011 audit of 2,241 US companies found only 37% responded to a web lead within an hour, and 23% never responded. A tracking map shows where leads wait and where they disappear.

What this looks like for elective practices

Your “client” is an accepted treatment plan, recorded in a practice management system such as Dentrix, Open Dental, Nextech or PatientNow. Those systems hold protected health information, which limits how far marketing data can travel. HHS guidance says tracking on patient portals, scheduling and other pages that transmit PHI is still covered by HIPAA, and vendors handling PHI need a business associate agreement (HHS OCR). Check every call tracking, form and chat tool.

The common leak sits between consult request and consult. A coordinator books by phone, the source stays blank, and the consult arrives with no channel attached.

Demand is there. ASPS reported on September 1, 2026 that cosmetic surgical procedure volume rose 7% in 2025. Many practices attribute at the booked-consult level and report accepted treatment as a count by source, with no patient detail leaving the practice system.

Step 1: Draw your lead path and test it

Start on paper. Write down every stage a lead passes through, the tool that holds it and the person who owns it. Many owners find a stage that lives in someone’s inbox.

Stage	Tool	Owner	Source carried through? (yes / no / unsure)
Click or call (search, map listing, social, referring doctor)			
Inquiry (form, call, chat, financing pre-check)			
Consult booked			
Consult held			
Won: treatment accepted, deposit or financing in place			

Every “no” or “unsure” is a leak. Past it, you cannot tell which channel deserves credit.

Now run the source test. Submit one test inquiry per channel, using a landing URL with UTM tags such as `?utm_source=google&utm_medium=cpc&utm_campaign=sourcetest` and a test

name you will recognize. Call each tracking number too. Then check where the source survives and delete the test records.

Channel	Date tested	Source in form tool?	Source in CRM?	Source in system of record?

Your system of record is the practice management system. Use test names with no health detail and no procedure of interest.

Step 2: Audit the tool stack

The goal is not cutting for its own sake. It is finding two tools doing one job, or an expensive tool kept for one feature.

Tool	Monthly cost	What we actually use it for	Overlaps with	Who logs in weekly?	Keep / cut / replace

- [ ] Two tools that both send appointment reminders
- [ ] A full-tier platform used for one form or one email
- [ ] A scheduling link that books into a calendar but never into the CRM
- [ ] Former staff or vendors who still have logins

Annual cost ÷ features you use = cost per feature. The outliers go to the top of the review list. Never cancel a tool that holds client records until the data is exported and has a new home.

Step 3: Build the first three integrations

Fix leaks in order of how much revenue passes through them. Check settings before paying for anything.

1. Form plus UTM into the CRM. Capture UTM values and the landing page in hidden fields, mapped to source fields, not a notes box.

2. Call tracking into the CRM. Each call should create or update a record with the caller, the source and the time. Native connections are common; a webhook covers

the rest.

3. Stage changes into reporting, and back to ad platforms where allowed. Booked, held and won should flow to your monthly report. Where permitted, send them to Google or Meta as offline conversions so the platforms learn from clients, not form fills.

For a practice, never send anything that could be PHI to an ad platform, and confirm a business associate agreement with every vendor that handles it. Many practices stop at counts in their own report.

This is general information, not legal advice. When each integration goes live, confirm test leads show the right source, duplicates merge, and someone is alerted when it fails.

Step 4: Decide whether to build or buy

Most fixes are settings, some are integrations, and a few justify a small build. Score each 1 to 5.

Factor	1 (buy or skip)	5 (consider building)	Your score
How often the task happens	A few times a year	Daily	
Staff hours per month on it	Under 2	Over 20	
Fit of the current tool	Cheap and fits well	Costly and fits poorly	
How unique your process is	Standard	Specific to you	
Risk if it breaks	Touches client or sensitive data	Internal only	

Twenty or more is worth a closer look. Under 12, buy or change a setting.

(Monthly hours saved × loaded hourly cost) + software cost removed = monthly value. Build cost ÷ monthly value = months to pay back.

Example numbers, not benchmarks. A coordinator spends 15 hours a month retyping inquiries into the system of record at $35 an hour: $525. If the integration also

replaces a $150 add-on, monthly value is $675, and a $4,000 build pays back in about six months, before maintenance.

Good builds are small: intake routers, source syncs, quote estimators, proposal generators. Review request triggers must follow the FTC's 2024 rule: no incentives tied to positive sentiment and no review suppression (Morgan Lewis, August 26, 2024). And fundamentals come first. If leads wait two days for a reply, no tool fixes that.

Step 5: Keep a one-page monthly report

The map only helps if you look at it. One page, once a month, by source.

Source	Spend	Leads	Consults booked	Held	Treatment accepted	Revenue	Cost per accepted treatment
Google Ads							
Google Business Profile							
Meta ads							
Organic search							
Doctor and patient referrals							
Total							

Cost per client = spend ÷ won. Booking rate = booked ÷ leads. Show rate = held ÷ booked. Close rate = won ÷ held.

Read the rates before the totals. A weak booking rate usually points to speed or follow-up, a weak show rate to reminders, and a weak close rate to the consultation itself.

Where do most owners get stuck?

APIs and webhooks break silently. A vendor renames a field or a token expires, and leads stop flowing with no visible error. Without monitoring, you find out weeks later from a gap in the report.

Knowing when not to build. The right answer is often a setting you did not know existed, or a tool you already pay for. That is easier to see from outside your own stack.

Knowing which vendors touch PHI is a project in itself. Call tracking, chat, forms and booking widgets each need checking, and some need a business associate agreement before they can stay.

If you want a second set of eyes on your map, book a 25-minute exploration call through the short form at the end of this guide.

Sources

- Harvard Business Review, "The Short Life of Online Sales Leads," March 1, 2011. link
- U.S. Department of Health and Human Services, Office for Civil Rights, "Use of Online Tracking Technologies by HIPAA Covered Entities and Business Associates," accessed October 15, 2026. link
- Morgan Lewis, "FTC Issues Final Rule on Consumer Reviews and Testimonials," August 26, 2024. link
- American Society of Plastic Surgeons, "Lift procedures led growth as demand for restoration drove plastic surgery in 2025," September 1, 2026. link

Want a second set of eyes on what you found?

Book a free 25-minute exploration call. We will look at your answers with you, name the first fixes in order, and tell you honestly if we are not the right fit.

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